Paclitaxel + cetuximab every 4 weeks

National Cancer Control Programme (NCCP) 00696 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first visit

Bring your medicines and allergy list, test dates, treatment calendar and actual emergency contacts. Tell the unit about swallowing, hearing, mobility, transport or communication needs.

STEP 2

Time in the unit

Cetuximab 500 mg/m² is infused on Days 1 and 15. The initial infusion has a slower maximum rate than later tolerated doses; total time depends on the prescribed dose. Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit. Paclitaxel is usually 80 mg/m² over 1 hour on Days 1, 8 and 15; a consultant may select a 100 mg/m² starting dose for some patients. The actual prescribed dose and premedicines must be confirmed. Day 8 is paclitaxel only.

STEP 3

Infusion and appointment planning

Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit. Day 8 is a shorter paclitaxel-only visit, but checks, preparation and observation still add time. Ask the unit for a realistic arrival and collection plan.

STEP 4

Before you leave

Check the next treatment and test dates, medicines, pump or line appointments, and who to contact if unwell. Have the team replace and verify every example phone number on the wallet card.

STEP 5

Practical support

Ask about transport, work, costs, caring responsibilities, an interpreter or a support person. Keep changes to the plan in one record.

Sources: NCCP 00696 v2: official regimen. Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending.